8.7 Hyperthyroidism and Thyroidectomy*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel hot and nervous and lose weight even though I eat." **Objective** • HR 118/min • Tremor • Lost 5 kg • Low TSH, high T4	**ACTUAL** Inadequate nutritional intake related to increased metabolic rate as evidenced by weight loss despite adequate intake.	• Weight steady within 4 weeks of treatment.	• Provide high-energy, high-protein meals. • Give the antithyroid drug and the beta-blocker as prescribed. • Weigh weekly.	• Considered met if weight is stable.
Subjective • After thyroidectomy: "It hurts to swallow." **Objective** • Neck incision • Hoarse voice	**ACTUAL** Acute pain related to the neck incision as evidenced by odynophagia.	• Pain at or below 3/10 within 24 hours.	• Sit her semi-Fowler with pillows supporting the neck. • Give analgesia as prescribed. • Offer cool, soft food. • Check her voice hourly at first; report increasing hoarseness.	• Goal reached when pain is controlled.
Subjective *No specific complaint* **Objective** • Neck surgery close to the parathyroids and airway	**RISK** Risk for physical injury as evidenced by risk factors: post-thyroidectomy hematoma, laryngeal nerve injury and hypocalcemia.	• Airway clear and calcium normal.	• Keep a tracheostomy set at the bedside. • Check the dressing and the back of the neck for bleeding. • Report tingling around the mouth or in the fingers, as this suggests low calcium. • Follow calcium.	• Met if no complications occur.

*References for this care plan: [32], [35], [37], [115]. Full entries are listed in the References section of the CPMN book.